Dambatta, Kano State, Nigeria

In Kano's dryland north, El Niño's deaths will come through the calendar, not the thermometer: meningitis from January, cholera, and child malnutrition in the 2027 lean season. Put treatment and drugs in place before each one starts, and use the dryland-investment money that already exists for the farms.

IMPACT REGION	EXCESS HEAT DEATHS	WINDOW	DOMINANT HAZARD
Not resolved. Dambatta LGA (207,968 people, 2006 census; ~732 km²; 11 districts), 79 km north of Kano city	Nigeria 31,400 (15,400 in Dec–Feb). Population-scaled to Dambatta: ~47, our arithmetic	6-month, Sep 2026 – Feb 2027	Meningitis (Dec–Jun), cholera, lean-season child malnutrition (Mar–Aug 2027); heat peaks Mar–May; drought and farmer–herder tension

16 recommendations: 2 confirmed, 9 corrected, 4 unverified, 1 contraindicated. All but two were checked by verifiers. **Why Dambatta:** asked for the part of Kano "most appropriate for desert farming", a scoping agent chose Dambatta. It lies on the Sahel–Sudan transition, with sandy soils and dryland groundnut, millet, sorghum and dry-season onion farming, and it is home to Audu Bako College of Agriculture. Kunchi and Tsanyawa were runners-up. Rainfall figures on Wikipedia for these LGAs are unreliable and were not used. Exposure figure: Climate Impact Lab, *451,000 at Risk*, September 2026, pre-peer-review; estimated with no African mortality data. **The ~47 figure is a**

* Suggested formulation: check with a health worker before sharing.

EXECUTIVE SUMMARY — AT A GLANCE



12.43°N 8.52°E, Nigeria · Natural Earth

- **The risk.** Deaths come on the dry-season calendar, through meningitis, cholera, and lean-season child malnutrition: the 1996 epidemic killed ~12,000 in northern Nigeria, and Kano was a cholera hotspot in 2014, 2018, and 2021.
- **First action.** The Kano State PHC Management Board, UNICEF, and Dambatta General Hospital scale up severe child-malnutrition treatment, stocking a 3–6 month therapeutic-food buffer by February 2027.
- **The money.** ~$145k floor, ~$440k target, ~$740k ceiling. The top placement is severe-malnutrition treatment and food buffer through the Kano PHC Board and UNICEF pipeline, $190,000.
- **Do not.** Do not wait for the meningitis epidemic threshold before acting: reactive campaigns averted only about 6% of cases in Nigeria and Niger in 2015–17, and in 2017 vaccination started nine weeks late.

What to do	**9** actions	p. 2
Who does what	RACI chart	p. 9
Where money does the most	funding	p. 9
Do not do this	**5** items	p. 11
What has happened here	the record	p. 12
How this was produced	methodology	p. 14
References	sources	p. 15

flat population scaling (207,968 ÷ 140.4 million × 31,400), our arithmetic, not an Impact Lab finding. Web search was unavailable; agents fetched sources directly, and nothing found was specific to Dambatta for any epidemic, drought or flood, so the record below is Kano-wide or national.

WHAT THIS PLACE SHOULD DO RANKED

Ranked by expected lives saved per unit of effort before and through the peak. Backing lenses are shown per action. The RACI chart below assigns each one.

1. Scale up treatment of severe child malnutrition at Dambatta's health centres before the 2027 lean season, with a 3–6 month buffer of therapeutic food by February

PARTIALLY CONFIRMED | PUBLIC HEALTH · PHILANTHROPIST · EMERGENCY · SKEPTIC | ADAPTATION INVENTORY: NOT CONSULTED

OWNER	BY WHEN	COST	PREVENTS
Kano State Primary Health Care Management Board; UNICEF for therapeutic food (RUTF); Dambatta General Hospital for complicated cases	Stock by Feb 2027; treat Mar–Aug 2027	~$190,000 (see funding)	Lean-season child deaths after a weak harvest

Volunteers screen children with arm-circumference (MUAC) tapes; children with severe malnutrition are treated at health centres with ready-to-use therapeutic food, and those with complications go to Dambatta General Hospital. Kano's under-5 mortality is 164 per 1,000, with 56.9% of children stunted and 6.4% wasted (DHS 2018, state level). UNICEF says only about 2 in 10 Nigerian children with severe malnutrition are reached. Community-based treatment costs $26–53 per DALY averted (2020 systematic review), and a trial in northwestern Nigeria put the provider cost, including the food, at $67.07 per child. FEWS NET projects Crisis-level food insecurity (IPC Phase 3) across much of northern Nigeria through January 2027, with staple prices well above average.*

Corrections: the FEWS NET projection is not stated for Kano specifically. Whether Dambatta has working outpatient treatment sites, and whether MSF has a site in Kano able to take referrals, were not verified. The death-rate difference between treated and untreated cases (~10 points) is an assumption, not a measured figure. Cash plus supplementary food (Langendorf 2014, Niger) is evidence for prevention

* Suggested formulation: check with a health worker before sharing.

in children aged 6–23 months, an optional add-on.

2. Meningitis: find out whether Dambatta ever received Men5CV, request preventive vaccination if not, and stock drugs and rapid tests before December

PARTIALLY CONFIRMED | PUBLIC HEALTH · EMERGENCY · SKEPTIC · HISTORIAN | ADAPTATION INVENTORY: NOT CONSULTED

OWNER	BY WHEN	COST	PREVENTS
Kano State PHC Management Board with the Dambatta LGA disease surveillance officer; NCDC and NPHCDA for the vaccine request	30 Nov 2026	~$8–12k for drugs and tests (lens estimate)	Deaths from late diagnosis, the wrong serogroup assumption, and late vaccination

Pre-position ceftriaxone and multi-serogroup rapid tests at Dambatta's health centres, and run weekly zero-reporting against WHO thresholds: alert above 3 and epidemic above 10 suspected cases per 100,000 a week. Waiting is what fails: reactive campaigns in 2015–17 averted about 6% of cases across Nigeria and Niger, and about 3% in Nigeria (Cooper et al. 2019). Preventive campaigns work: MenAfriVac produced a 94% difference in incidence in Chad. In Gusau, Zamfara (2010), the dominant serogroup had shifted to W135 and none of 122 spinal-fluid samples grew on culture, so plans that assume serogroup A miss the epidemic.

* Suggested formulation: check with a health worker before sharing.

Corrections: an emergency lens gave the alert threshold as 10; that is the epidemic level. Kano's inclusion in any Men5CV campaign could not be confirmed; the only documented Nigerian use is a reactive campaign in three Yobe LGAs, and WHO dates the first Gavi preventive campaign to Niger in 2025. A verifier disputed the Nigeria-only 3% figure; it rests on the paper's own country breakdown (270 of about 10,000 cases), read directly in the Kakuri run.

3. Cholera: one oral rehydration point per district, chlorine tablets, testing of water sold by vendors, and treatment kits in stock before the 2027 rains

PARTIALLY CONFIRMED | EMERGENCY · PUBLIC HEALTH · SKEPTIC · HISTORIAN | ADAPTATION INVENTORY: NOT CONSULTED

OWNER	BY WHEN	COST	PREVENTS
Nigerian Red Cross Kano branch with the Dambatta LGA WASH unit; Kano State Ministry of Health; NCDC for any vaccine request	Points by 15 Dec 2026; full readiness before Jun 2027	~$15k (lens estimate)	Deaths from delayed rehydration and unsafe vendor water

Nigeria has had more than 50,000 cholera cases and 338 deaths so far in 2026 (FEWS NET). Nationally, 1991 saw 59,478 cases and 7,654 deaths, and 2021 saw 111,062 cases and 3,604 deaths, with Kano among the hotspots in 2014 and 2018. A Kano outbreak in 1995–96 was traced to unsafe water sold by street vendors, so test vendor supply. Where ORS is promoted, diarrhoea deaths fall about 69% (Munos et al. 2010), and the same points serve ordinary child diarrhoea.

* Suggested formulation: check with a health worker before sharing.

Corrections: the 2026 outbreak is driven mainly by Borno, not Kano. The vendor-water link is cited from a review, not a primary study. Cholera vaccine cannot be stockpiled locally (see Contraindicated).

4. Reach every household through the traditional chain, the Friday mosque, and women who can enter compounds

PARTIALLY CONFIRMED | FRONTLINE · SKEPTIC | ADAPTATION INVENTORY: NOT CONSULTED

OWNER	BY WHEN	COST	PREVENTS
District head (Hakimi), village heads (Dagaci) and ward heads (Mai Unguwa); imams; female volunteers from the School of Basic Midwifery and FOMWAN	Briefing by 30 Nov; messages Dec–Jun	Volunteer time	Warnings and services missing half the households

Start at the district head's palace, not the LGA chairman: ward heads knock on doors when the palace asks them to. Imams give one line each Friday from December to June: stiff neck with fever means the clinic today; boil or chlorinate water; check on older people and children at midday. Female volunteers visit compounds where women live in seclusion (kulle) and men cannot enter, carrying ORS instructions, signs of heat illness in infants, and MUAC tapes. Islamiyya and Tsangaya teachers report sick almajiri children. Freedom Radio (99.5 FM, Kano) carries the same messages in Hausa.*

Corrections: Dambatta moved back from the abolished Bichi Emirate to the Kano Emirate on 23 May 2024, and the emirate dispute is still before the Supreme Court (hearing postponed to 2027), so confirm the current district head at the LGA secretariat. Freedom Radio's reach into Dambatta was not confirmed. Evidence that female health workers raise uptake comes from a four-country vaccine study, not Kano; a

* Suggested formulation: check with a health worker before sharing.

2026 Kano study shows husbands and mothers-in-law decide on care, which is why women must be reached directly. The 1996 Pfizer Trovan trial in Kano (about 200 children; $75m settlement in 2009) still shapes trust in health interventions, so who delivers the message matters.

5. Get Dambatta into Kano's World Bank dryland programme (ACReSAL) before it closes in March 2028, and pilot paid farmer-managed tree regeneration

PARTIALLY CONFIRMED | ECONOMIST · HISTORIAN · SKEPTIC | ADAPTATION INVENTORY: NOT CONSULTED

OWNER	BY WHEN	COST	PREVENTS
Kano State ACReSAL project unit, with Kano State Forestry and KNARDA; Dambatta LGA and district head to request sites	Request sites before the 2027 work plan	Inside ACReSAL's $700m programme	Losing farmland, wells and livelihoods to the next dry run of years

This is the desert-farming lever. ACReSAL (World Bank P175237, $700m, active to 31 March 2028) works in 19 states including Kano, where its results framework lists 36,317 ha under sustainable land management, about 1.65 million beneficiaries, and solar boreholes. Its Kano sites are in Makoda, Takai, Shanono, Ungogo, Dawakin Tofa, Tarauni and Kiru, not Dambatta. Ask for Dambatta sites: solar boreholes for dry-season irrigation and livestock, and a pilot paying farmers for trees still alive after three years. In Maradi, Niger, farmer-managed natural regeneration started with about 10 farmers in 1983 and covered more than 5 million hectares, half of Niger's farmland, by 2004. The Great Green Wall had covered only 4% of its planned area by 2020.

* Suggested formulation: check with a health worker before sharing.

This helps 2027–28 and beyond, not this season's deaths. No Nigerian or Kano evidence for farmer-managed regeneration was found, and outcomes of Dambatta's own shelterbelt programme could not be verified, so the report makes no claim about it. Trees on cropland can provoke disputes with herders, so pair any pilot with action 6.

6. Heat readiness for March–May: shade, water, ORS and skin-wetting at health facilities, and warnings through the traditional chain *

PARTIALLY CONFIRMED | EMERGENCY · SKEPTIC · PUBLIC HEALTH | ADAPTATION INVENTORY: NOT CONSULTED

OWNER	BY WHEN	COST	PREVENTS
Kano State Emergency Management Agency (SEMA), relaying NiMet forecasts; PHC Board for facilities	Live by 1 Feb 2027	Under $5k (lens estimate)	Heat illness in children under five, pregnant women and older people

Prioritise children under five and pregnant women as well as the elderly; in the one Sahelian cohort, under-5 deaths rose 3.7% per 1 °C (Diboulo et al. 2012, Burkina Faso). Pair every heat message with safe water and ORS, since heat and cholera both peak when water is short.

Fans are no help above 35 °C (Meade et al. 2024), and Kano's hot season exceeds that, so the guidance is shade, wetting skin and clothing, and moving to cooler places, not fans.

7. Ask the Nigerian Red Cross why Nigeria's pre-funded drought protocol (MDRNG044) is frozen, and whether it covers Kano or Jigawa

PARTIALLY CONFIRMED | ECONOMIST | ADAPTATION INVENTORY: NOT CONSULTED

OWNER	BY WHEN	COST	PREVENTS
Nigerian Red Cross Kano branch and national HQ; IFRC decides	This month	A phone call; no new money	The only pre-arranged drought money in Nigeria sitting unused

IFRC's drought protocol for Nigeria is fully funded (CHF 218,996, 3,224 people, March 2026 – February 2028) and marked "Frozen". No document says which states it covers or what its trigger is.

* Suggested formulation: check with a health worker before sharing.

Do not describe it as covering Kano. Its size is small: about $68 per person for 3,224 people.

8. Enrol farmers in subsidised crop insurance for the 2027 season

PARTIALLY CONFIRMED | ECONOMIST | ADAPTATION INVENTORY: NOT CONSULTED

OWNER	BY WHEN	COST	PREVENTS
Nigerian Agricultural Insurance Corporation (NAIC), Kano office; Dambatta LGA extension staff for enrolment	Before 2027 planting (May–Jun)	Farmers pay half the premium; premium not published	A failed harvest wiping out a household's savings and seed

NAIC offers a 50% subsidy on premiums for selected crops and livestock. NIRSAL separately guarantees up to 75% of agricultural loans. No weather-index product exists for Kano, so cover pays after a loss is assessed, not on a trigger.

Correction: a link between NAIC cover and NIRSAL loans is not documented, so they are two separate tools. Insurance pays after the harm, so this protects next year's planting, not this season's lives.

Unverified: leads to check before acting

UNVERIFIED

A weekly burial count from village heads and gravediggers to the LGA surveillance officer, from now. Kano's 2020 death spike was reportedly first noticed through burials, but that could not be verified, and the precedent for community death counts in Nigeria is thin. **A cattle-route and boundary meeting** between Fulani camp heads (Ardo), village heads and onion farmers before dry-season irrigation, convened by the district head. The Mai Gamji clash of November 2023 left many farmers injured and one reported dead, but the meeting's effect is untested. **Registration on the National Social Register** for lean-season cash: the World Bank-funded National Social Safety-Nets Project scale-up is live, paying three tranches of ₦25,000, and had reached 9.18 million households by 31 March 2026 with about $55 million undrawn (Daily Trust, March 2026). Whether Dambatta households are on the register, and when payments land, was not verified. It is a small top-up, not protection. *Corrected 4 October 2026: an earlier version said no live programme was found.* **Seasonal malaria chemoprevention for the full four cycles in July–October 2027:** who delivers it in Kano could not be confirmed.

* Suggested formulation: check with a health worker before sharing.

WHO DOES WHAT

RACI CHART

R = Responsible (does the work) · **A** = Accountable (one per action; signs off and answers for it) · **C** = Consulted (asked before) · **I** = Informed (told after). Built from the owners named in the verified actions. No one here has been contacted or has agreed to a role; every row is a proposal.

Action	Kano SEMA	Kano PHC Board	Kano Min. of Health	NCDC / NPHCDA	Dambatta LGA	Dambatta General Hospital	District & village heads	Imams	Women volunteers (midwifery school, FOMWAN)	Nigerian Red Cross (Kano)	UNICEF	ACReSAL Kano unit / Forestry / KNARDA	NAIC	IFRC	NiMet	Herder leaders (Ardo)
1. Severe malnutrition treatment	I	A	C		R	R	I	I	R	C	R					
2. Meningitis readiness and vaccine request	I	R	A	C	R	R	I	I		I						
3. Cholera points, chlorine, vendor testing	I	R	A	C	R	C	I	I	I	R						
4. Traditional chain, mosque, women's outreach	I	C			I		A/R	R	R	C						
5. ACReSAL sites and tree-regeneration pilot	I				R		R					A				C
6. Heat readiness, Mar–May	A	R	C		I	R	I	I	I	R					C	
7. Unfreeze the drought protocol	C		I							R				A		
8. Subsidised crop insurance					R		C					C	A			

What the chart shows. **Health accountability is split between the Kano State Ministry of Health and the PHC Management Board**, so one joint meeting covers the three actions that save the most lives. **The district head is the only party both accountable and doing the work** (action 4), which is the frontline lens's point: the palace is the front door. **The two farming actions sit with bodies outside health altogether** (the ACReSAL unit and NAIC), so they need their own owner on the team.

WHERE MONEY DOES THE MOST

FUNDING

The Philanthropist lens, reconciled against the ranked actions and verified. Placements are ranked by marginal lives saved per dollar. The envelope is bounded by what this place can spend before the peak, not by what it needs. Nothing pre-arranged is confirmed for Dambatta: the drought protocol is frozen with unknown coverage, IFRC's 2026 cholera operation covers Borno only, and the national cash-transfer scale-up is live but pays on a schedule, not a trigger, and its reach into Dambatta is unverified. *Corrected 4 October 2026: an earlier version said the programme closed in December 2025.*

FLOOR	TARGET	CEILING	LARGEST UNCERTAINTY
$145,000	$440,000	$740,000	How many children will need malnutrition treatment, and whether

* Suggested formulation: check with a health worker before sharing.

					UNICEF or MSF already covers Dambatta

Malnutrition caseload: 207,968 × 1.03^{20} ≈ 375,600 people in 2026 (3% annual growth, our assumption) × 20% under five ≈ 75,000 × 3% annual incidence of severe malnutrition (an unsourced assumption; prevalence-to-incidence conversion suggests 2.6–3.9%) ≈ 2,250 children. Cost: $67.07 per child (2019, northwestern Nigeria trial, food included) × ~1.25 for inflation (our assumption) ≈ $84 × 2,250 ≈ $190,000. Floor = half of that ($95,000, if others cover the rest) + $50,000 Red Cross. Target = $190,000 + $100,000 Red Cross + $150,000 malaria chemoprevention (held). Ceiling = $190,000 + $100,000 + $450,000 for a full chemoprevention season (75,000 × $6). The funder lens proposed $520,000; verification removed a double count of the food cost (−15%, under the one-third threshold). If incidence is 50% higher, the malnutrition line rises to about $285,000.

RECIPIENT	USE	AMOUNT	MECHANISM	BASIS	VERDICT
Kano State PHC Management Board / UNICEF pipeline (Red Cross Kano as fallback)	Action 1: severe-malnutrition treatment and food buffer	$190,000	Therapeutic food purchase plus top-up for running sites	$67.07 per child incl. food (northwestern Nigeria trial, 2022); $26–53 per DALY (2020 review)	Partially confirmed; confirm sites and food gap with the Board or UNICEF before release
Nigerian Red Cross, Kano branch (1 Nasarawa Hospital Road, Kano)	Actions 3, 4, 6: rehydration points, case-finding volunteers, heat messaging	$100,000	Direct grant in tranches	No published cost per death	Confirmed (branch details); no record of spending at this scale
Kano malaria chemoprevention programme (deliverer unconfirmed)	Full four cycles, Jul–Oct 2027	$150,000 (held)	Gap top-up	~$1.50 per cycle (GiveWell); $2,000–7,000 per death averted, not a Kano figure	Unverified; only if a gap is confirmed
Outside the target: New Incentives	Routine childhood immunisation incentives, Kano State	Optional	Through GiveWell or New Incentives	$1,500–6,000 per death averted; Dambatta coverage unknown	Confirmed in Kano; off-mechanism for El Niño

* Suggested formulation: check with a health worker before sharing.

The funder's one thing: one phone call sets most of this envelope. Ask the PHC Board's nutrition desk, or UNICEF's Kano office, before 1 November how many outpatient malnutrition sites operate in Dambatta and how many months of therapeutic food are in stock for the lean season. Then fund the food gap, not new buildings.

DO NOT DO THIS — CONTRAINDICATED

Advice that circulates for this hazard and would cost lives here.

"Pre-position cholera vaccine in Dambatta"

CONTRAINDICATED AS WRITTEN

Oral cholera vaccine is released from the global stockpile through the International Coordinating Group and Gavi, on a national request from NCDC. It cannot be held at LGA level, and planning as if it could leaves a gap when the outbreak starts.

Instead: pre-position treatment kits, ORS and chlorine locally, and get Dambatta onto NCDC's hotspot list so a vaccine request moves fast.

Waiting for the meningitis epidemic threshold before acting

CONTRAINDICATED

Reactive campaigns averted about 6% of cases in Nigeria and Niger in 2015–17, because vaccination started weeks after the threshold. In 2017 it started nine weeks late.

Instead: confirm Men5CV coverage now, stock drugs and tests, and request preventive vaccination before the harmattan (action 2).

Fans as heat advice, and "drink more water" without safe water*

CONTRAINDICATED

Fans don't help above 35 °C (Meade et al. 2024), and Kano's March–May heat goes above that. Telling people to drink more water where the water is unsafe spreads cholera.

* Suggested formulation: check with a health worker before sharing.

Instead: shade, wetting skin and clothing, and "treat or boil water, keep it covered, start ORS early".

Counting seedlings planted as success

CONTRAINDICATED AS A MEASURE

Planting programmes report seedlings handed out, not trees alive. The Great Green Wall covered 4% of its planned area by 2020, while farmer-managed regeneration in Niger covered half of the country's farmland.

Instead: pay for trees still alive after three years, and protect what farmers already manage (action 5).

Delivering only through compound heads, market days or men

CONTRAINDICATED

Women in seclusion are not reached, and husbands and mothers-in-law make care decisions. A male-only channel reaches half the households.

Instead: female volunteers house to house, with any cash registered in women's names (action 4).

WHAT HAS HAPPENED HERE BEFORE — THE RECORD

YEAR	EVENT	TOLL	WHAT CHANGED AFTER
1968–73 (El Niño 1972, ONI +1.8)	Great Sahelian drought	Up to 100,000 dead across the Sahel; about a third of livestock	Farmers built coping strategies
1983–84 (El Niño, ONI +2.1)	Sahel drought, worse rainfall deficit	Fewer deaths than 1972–74	Credited to adaptation on farms
1996	Meningitis epidemic; Pfizer Trovan trial in Kano	~12,000 deaths in northern Nigeria; ~200 children in the trial	$75m settlement (2009); lasting distrust of health interventions

* Suggested formulation: check with a health worker before sharing.

YEAR	EVENT	TOLL	WHAT CHANGED AFTER
2010	Meningitis, Gusau (Zamfara)	Serogroup W135 dominant; 0 of 122 cultures positive	Showed plans assuming serogroup A fail
2014, 2018, 2021	Cholera; Kano a hotspot	2021 nationally: 111,062 cases, 3,604 deaths	Response mobilised after case surges
Nov 2023	Farmer–herder clash, Mai Gamji, Dambatta	Many farmers injured; one death reported	Not documented
May 2024	Bichi Emirate abolished; Dambatta back under the Kano Emirate	—	Emirate dispute still in court
2026	Cholera surge (mainly Borno); Crisis-level food insecurity projected across the north	>50,000 cases, 338 deaths nationally	No Kano-specific response found

The recurrence nobody has noticed: the 1983–84 drought had worse rainfall than 1972–74 and killed fewer people, because farmers in the Kano Close-Settled Zone had adapted their tree, manure and land management (Mortimore's long-run work). What saved lives was what farmers already did, not what projects delivered. Protect that before planting anything new.

WHERE THE EXPERTS DISAGREE — UNRESOLVED

- **Spend about half a million dollars, mostly on malnutrition treatment** (Philanthropist) versus **the cheapest lever is a phone call to unfreeze the pre-funded drought protocol** (Economist). Both are kept: the call costs nothing and comes first; the treatment budget depends on a caseload that is still an assumption.
- **Plant trees and shelterbelts** (the region's standard approach) versus **pay for regeneration measured by survival** (Skeptic, Historian). The report recommends a paid pilot, not a replacement, because no Nigerian evidence exists either way.
- **Heat is the headline** versus **the killers here are meningitis, cholera and hunger, on the dry-season calendar** (every lens). Heat readiness is kept, ranked sixth.

* Suggested formulation: check with a health worker before sharing.

WHAT NOBODY COVERED

BLIND SPOT

Two things. First, **nobody could see the services on the ground**: whether Dambatta has outpatient malnutrition sites, therapeutic food in stock, Men5CV coverage, or who delivers malaria chemoprevention. Those four facts set most of the plan and the money. Second, **the desert-farming question runs on a different clock**. Irrigation, boreholes, tree regeneration and insurance protect 2027–28 harvests; they save few lives before this season's peak. If the purpose behind choosing Dambatta is dryland farming, that deserves its own study of water tables, onion irrigation and soils, which this briefing does not attempt.

HOW TO READ THIS

METHOD

The panel is author-built. Seven simulated expert lenses were used to generate these recommendations. Where they agree, that is a strong hypothesis, not a consensus of the field. Every recommendation was then independently checked for evidence, transferability to this location, and feasibility before the peak; the verdict tags carry the result.

Mortality figures are resolved at Climate Impact Lab impact-region scale, roughly 300,000 people. They are not neighbourhood figures. The El Niño projection covers *heat only*; drought, flood, fire, smoke, and crop failure are additional to it and are carried here by the historical record rather than by a published number.

HOW THIS BRIEFING WAS PRODUCED

METHODOLOGY

- **Scope.** Angus asked for "Kano, Nigeria", then for the area "most appropriate for desert farming". A scoping agent compared northern Kano LGAs (Kunchi, Tsanyawa, Makoda, Dambatta, Bichi) and chose Dambatta LGA; Angus approved the choice in advance. Climate Impact Lab impact region, as the report records it: not resolved. Mortality figures exist only at that scale (impact regions average roughly 300,000 people); sub-city detail is used for hazard exposure and action design, not for death counts.
- **Exposure.** Climate Impact Lab, *451,000 at Risk* (September 2026, pre-peer-review): ECMWF SEAS5 seasonal forecasts (51-member ensemble) applied to temperature–mortality relationships across 24,378 regions, against a 1996–2025 baseline, for June 2026 – February 2027. It covers heat only. It is never added to the Impact Map's long-run projections or quoted from the map's GDP-denominated mortality. Where heat is not the dominant hazard, the historical record carries the hazard instead of a borrowed number.
- **Expert panel.** Seven simulated expert lenses (emergency manager, public health scientist, frontline responder, skeptic, economist, historian, and philanthropist) each researched this place independently and in parallel, from web sources, to a fixed brief.

* Suggested formulation: check with a health worker before sharing.

- **Reconciliation.** Duplicate actions were merged and backing lenses recorded. Convergence was tested for shared bias (whether the evidence behind an agreed action came from a setting like this one) before it counted in an action's favour. Contradictions were kept, not resolved by dropping a side. Actions were ranked by expected lives saved per unit of effort before the peak, each was given a named payer and trigger or marked unfunded, and the funding placements were reconciled against the ranked list. The RACI chart was built from the owners named in the verified actions.
- **Verification.** Five verifier agents checked the recommendations on three tests: evidence (the primary source and its effect size), transferability (whether that evidence came from somewhere like here), and feasibility (whether it can be done here before the peak). Financial actions were also checked for availability in this jurisdiction, a named payer, a trigger threshold, and whether they already exist. The funding envelope was checked for real recipients, published unit costs, and duplication, then re-summed.
- **Verdicts.** *Confirmed*: evidence, transferability, and feasibility all hold. *Partially confirmed*: holds after the corrections shown. *Unverified*: could not be checked to a primary source. *Contraindicated*: would do harm or waste the window here.
- **Adaptation Inventory.** Not consulted: web search was exhausted for this session, and its download sits behind a sign-up form.
- **Health wording.** Passages where a wording error could hurt someone (malnutrition treatment, danger signs and when to go to the clinic, water treatment and ORS, and cooling and fans) are marked "* Suggested formulation: check with a health worker before sharing." They were checked against published sources, not by a health professional. They are suggestions, not medical advice, and anyone passing them on should confirm them with a doctor, pharmacist, nurse, or community health worker first. The marker was added on 2 November 2026.
- **Rules.** No cost is derived by dividing a published total, and no local figure is invented; where data is thin, the report says so and shows the containing geography's number with its arithmetic. Every source is listed below with its verification status.
- **Limits.** The panel and verifiers are AI research agents working from web-available sources, not human experts, and no one in this place was interviewed. Run-specific gaps: web search unavailable; no Dambatta-specific epidemic, drought or flood record found; Wikipedia rainfall figures for northern Kano LGAs are unreliable; Kano's Men5CV status, outpatient malnutrition sites, SMC deliverer, the drought protocol's states and trigger, Kano SEMA's budget, and the current district head's name were not verified; the malnutrition caseload rests on two stated assumptions.

References · Malnutrition: NDHS 2018 Kano, CMAM cost trial (PMID 36962786), CMAM cost per DALY review, Langendorf et al. 2014, UNICEF Nigeria nutrition, MSF Nigeria, FEWS NET Nigeria (confirmed, corrected) · Meningitis: Cooper et al. 2019, Gusau 2010, WHO meningitis fact sheet, Daugla et al. (PMID 24035220), Abdullahi v. Pfizer (confirmed, corrected) · Cholera: Nigeria cholera review, Munos et al. 2010 (confirmed) · Heat: Diboulo et al. 2012, Meade et al. 2024 · Place and community: Dambatta, Bichi Emirate, Mai Gamji clash, Freedom Radio, Ebinim et al. 2026 (confirmed,

* Suggested formulation: check with a health worker before sharing.

corrected) · Drylands: ACReSAL Kano, Farmer-managed natural regeneration, Great Green Wall, Michael Mortimore, Sahel drought (confirmed; Dambatta shelterbelt outcomes unverified) · Finance: IFRC MDRNG044, NAIC, NIRSAL, Nigerian Red Cross branches, GiveWell on New Incentives, GiveWell on SMC · ENSO: NOAA ONI

Exposure data · Climate Impact Lab, 451,000 at Risk · Impact Map · Adaptation Inventory

Generated 2 October 2026 for Dambatta LGA, Kano State, Nigeria; cash-transfer status corrected 4 October 2026. El Niño Ready is volunteer-run and is not affiliated with Climate Week NYC, The Climate Group, or Columbia University.

* Suggested formulation: check with a health worker before sharing.